June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Submitted electronically via https://www.regulations.gov

RE: Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Health Gorilla appreciates the opportunity to comment on CMS's Request for Information (CMS–0042–NC) regarding interoperability and the Trusted Exchange Framework and Common Agreement™ (TEFCA). As one of the first Qualified Health Information Networks (QHINs) under TEFCA, Health Gorilla brings extensive, real-world experience connecting disparate health IT systems—including major electronic health record (EHR) platforms, value-based care organizations, labs, and pharmacies, as well as other digital health innovators. Our policy-driven exchange platform processes hundreds of thousands of daily queries and has helped shape best practices in identity-proofing, data quality, and seamless workflow integration. We submit these comments to share practical insights on advancing patient and provider access, reducing administrative burden, and fostering a competitive, policy-driven ecosystem. In developing our responses, we drew on our partnerships with major EHR vendors, digital health innovators, and value-based care partners—particularly around risk stratification, attribution workflows, and quality-metric reporting—to ensure that our recommendations not only address broad interoperability goals but also align with the real-world needs.

Below is an outline of the specific sections addressed in our submission:

- **Patient and Provider Access (PC-10 through PC-14):** Input on TEFCA implementation, unified provider directories, seamless booking, transparent cost estimates, and lightweight "report issue" workflows for capturing both structured and unstructured exchange failures.
- **Operational Data Use Cases (PC-12):** Recommendations for API-driven care navigation (same-day directory lookups, scheduling, cost-estimate integration) and fostering a competitive marketplace on quality and convenience.
- **Information-Blocking Enforcement (PR-12 through PR-14):** Best practices for structured FHIR-based complaint reporting, centralized triage, and dashboards that close the feedback loop and drive continuous improvement.
- **Digital Identity and Directory Services (PR-9, PR-10):** Challenges, benefits, and implementation strategies for credentialing services, on-demand provider directories, and standardized trust frameworks.
- **Unstructured Data Access (PR-3):** AI-augmented ingestion pipelines that OCR and normalize scanned documents, faxes, and free-text notes into FHIR resources—plus unified error logging for end-to-end visibility.

- **Clinical Quality Reporting (PR-7, PR-8):** Simplified Bulk FHIR workflows, registry requirements, and pilot grant models to validate registry-driven quality metrics at scale.
- **API Certification & Value-Based Care Support (TD and VB series):** Criteria for payer, provider, and vendor endpoints—enabling on-demand risk stratification, automated attribution lists, and near-live performance dashboards to power VBC initiatives.

Our responses draw on direct experience with TEFCA policy implementation, on-demand FHIR API development, and close collaboration with EHR partners, value-based care organizations, labs, and pharmacies, as well as other digital health innovators. We trust these updated recommendations—reflecting partner feedback —will assist CMS and ONC in refining standards, incentives, and enforcement mechanisms to accelerate truly nationwide, patient-centric interoperability.

We thank you again for the opportunity to comment on this request for information. Health Gorilla commends CMS's dedication to fostering a more secure, efficient, and patient-centered health information ecosystem, and we appreciate the chance to share our firsthand experience. We remain committed to collaborating with your team to address emerging interoperability challenges, advance data quality standards, and streamline care coordination nationwide.

Respectfully submitted,
Health Gorilla

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Health Gorilla Comments

B. Patients & Caregivers

PC–10. How is the Trusted Exchange Framework and Common AgreementTM (TEFCA) currently helping to advance patient access to health information in the real world?

a. Please provide specific examples

TEFCA has markedly improved patient access by eliminating the need for separate portal credentials at each provider. Through a well-defined identity-proofing standard and a unified access interface, patients can now retrieve their health data in one place. For example, most of Health Gorilla's EHR participants are

fully configured to respond to TEFCA patient access requests, ensuring a consistent and secure experience across each system.

b. What changes would you suggest?

A number of organizations still leverage a "local policy exception" to decline TEFCA-based queries and revert to per-site logins—citing fears of OCR breach penalties if data is accidentally disclosed. To close this loophole, we recommend that CMS and ONC partner with OCR to establish a safe-harbor for any participant that (1) completes IAL2 identity vetting and (2) routes patient-requested data through a TEFCA-defined IASP. This would align enforcement expectations and remove the risk-aversion barrier to universal query compliance.

c. What use cases could have a significant impact if implemented through TEFCA?

Expanding patient-initiated queries via TEFCA could replace paper-and-CD transfers with FHIR-enabled digital delivery, enable patients to flag or annotate suspected record errors before transitions of care, and streamline downstream sharing under patient control—whether for clinical trial enrollment, secondary research, or life-insurance underwriting.

d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?

The IAL2 identity-proofing process under the CARIN framework is already helping ensure that patients are properly verified before accessing their records. Likewise, the HL7 FHIR Patient Access API delivers data in a consistent format across various systems, making the patient experience smoother.

e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?

The CARIN Alliance's federated identity broker model can streamline patient access by delegating identity proofing and consent workflows to trusted third parties. By adopting this approach more broadly, patients would face fewer sign-on hurdles and could seamlessly retrieve their records across disparate systems, enabling a more robust ecosystem of patient-facing applications.

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

There are no viable alternatives outside of TEFCA. HIEs remain essential local access points, but lack the nationwide scale and standardized APIs that TEFCA provides. Regional networks and health data utilities each operate under their own policies and rules, and national frameworks like Carequality and CommonWell treat patient access as optional—resulting in low provider participation. Meanwhile, individual EHR vendors have launched their own patient portals for records within their systems, spawning "Yet Another Portal" (YAP) syndrome and "portalitis," where patients must manage and remember too

many disparate credentials. Only TEFCA combines mandated data sharing, a unified trust framework, and guaranteed query responses at scale to deliver truly seamless, nationwide patient access.

PC–11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?

In most cases, data surfaced through HIEs matches the quality of records obtained via direct interfaces with source systems—labs, EHRs, and imaging repositories. The notable exception is patient matching: without a consistent, industry-wide matching standard, stewardship quality varies widely. Some HIEs employ dedicated data stewards and robust matching algorithms, while others rely on minimal rules, resulting in mismatch rates exceeding 1%. Given the volume of daily patient interactions, even a 1% error rate can lead to thousands of misattributed records each month.

b. What changes would you suggest?

To ensure consistent patient access and reduce fragmentation, we recommend CMS encourage and potentially incentivize HIEs (or health data utility iterations) to formally join the TEFCA framework. Participation would standardize identity proofing, consent handling, and query routing via Individual Access Service Providers, enabling uniform rollout of patient-initiated access use cases across regions and minimizing variability in technical implementation.

c. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?

While most HIE value remains inherently local—tailored to community needs—two organizations have demonstrated scalable, sustainable growth beyond their initial service areas: CRISP (Maryland/DC) and Contexture (formerly manifest in Indiana). Both have balanced financial viability with expanding provider participation and technical robustness. Rather than applying their playbooks wholesale, other regions should study their governance structures, funding models, and stakeholder engagement strategies to inform locally adapted implementations.

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

HIEs serve a critical local purpose and should be incorporated into broader data-exchange frameworks—particularly TEFCA—by acting as regional nodes within a unified trust architecture. This integration allows HIEs to continue managing community-specific policies and patient relationships while leveraging TEFCA's standardized security, consent, and technical protocols for seamless interoperability at scale.

PC–12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

Patients and caregivers need robust tools to find and engage the right providers by filtering on factors such as location, service offerings (in-person vs. virtual), language, cultural competence, insurance coverage, network participation, and real-time schedule availability. They also require seamless registration and appointment booking workflows, as well as transparent, individualized cost estimates—both retail and insurance-adjusted—so they can compare options and plan financially before receiving care.

b. What use cases are possible today?

Most provider and service information today appears on static websites, limiting discoverability and comparability. Once enrolled, patients may access additional data through portal systems, but this model restricts real-time visibility for those seeking new or alternative care options.

c. What should be possible in the near future?

Establishing patient- and caregiver-focused interoperability standards—enforced through HIT certification and linked CMS payment incentives—would ensure consistent access to core functionalities such as provider directories, scheduling availability, cost calculators, and quality metrics. A defined maturity model and implementation roadmap could guide health IT vendors and care organizations through successive feature rollouts in a transparent, technology-neutral manner that drives innovation and competition.

d. What would be very valuable but may be very hard to achieve?

Achieving full price and out-of-pocket cost transparency remains challenging due to fragmented payer systems and complex benefit designs. Launching a certification program for payer IT platforms, supported by CMS policy directives, would accelerate standardized pricing disclosures and empower patients with clear, personalized cost information.

PC–13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

CMS can launch a targeted, multi-channel education campaign to inform patients and caregivers of their legal rights to access, exchange, and use health data, setting clear expectations around the ease and timeline of obtaining records. The campaign should feature real-world examples of obstacles faced by individuals and proxies to highlight the importance of reporting suspected information blocking. To lower the barrier to entry, CMS or ONC could introduce a dedicated, patient-friendly reporting interface—either a streamlined web portal or mobile app—allowing users to submit complaints in just a few clicks.

HHS can then prioritize investigations by analyzing complaint volumes linked to specific workflows, providers, or health IT vendors, and publish enforcement summaries or transparent dashboards that showcase outcomes. Visible

follow-through will reinforce the value of reporting, motivate further submissions, and create a virtuous cycle of accountability. Although higher complaint volumes may introduce additional administrative work, robust investigation and public disclosure will ultimately drive stronger compliance, enhance data exchange, and rebuild patient trust in interoperability efforts.

PC–14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 credentialing service providers (CSP)):

a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?

The primary barrier is cost: leveraging credentialing services like CLEAR or ID.me incurs transaction fees—and sometimes subscription charges—which conflicts with CMS's mandate that patient-access APIs remain free. In practice, this forces many organizations to absorb identity-proofing expenses or revert to basic username/password flows, perpetuating a fractured patient-access landscape. Without a sponsoring entity willing to underwrite both the proofing and the secure data storage that follows, uptake among patients and caregivers remains limited.

b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

If more patients and caregivers adopted IAL2/AAL2 credentials, they would enjoy seamless, one-time sign-on across all certified portals and apps—eliminating repeated registrations and password resets—while benefiting from stronger, multi-factor proofing that guards against unauthorized access. A single, high-assurance credential would streamline aggregation of records from multiple providers into a unified view, empowering users and their care teams to coordinate treatment more effectively. Improved security and reduced friction would likely boost portal utilization and proactive health management, all while cutting down on help-desk calls and manual identity-proofing workflows.

d. How would encouraging the use of CSPs improve access to health information?

Encouraging the use of credential service providers would strengthen trust in digital health data exchange by ensuring that only properly vetted individuals can access sensitive information. With verified identity credentials in place, stakeholders—including patients, providers, and payers—can have greater confidence that data is being shared securely and with the appropriate person, which is foundational to expanding access and reducing hesitancy around digital health interactions.

e. What role should CMS/payers, providers, and app developers have in driving adoption?

Payers—both CMS and commercial insurers—should lead by funding credentialing costs for their covered populations, removing the financial hurdle for patients. Providers must prepare their front-line staff and workflows to manage increased inquiry volumes, and leverage the patient's own data to streamline intake processes rather than asking for repetitive history details. App developers, in turn, must safeguard patient

trust by implementing robust data-protection measures and offering transparent disclosures about how health information will be used.

C. Providers

PR–3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

Many of the barriers to accessing, tagging, and exchanging unstructured EHR content—such as narrative notes, scanned reports, and faxed records—can be overcome with AI-driven solutions that automatically identify, metadata-tag, and index critical data elements (for example, cardiac ejection fraction or other highly sensitive findings). Because neglecting unstructured data undermines even the best structured-data exchanges, prioritizing interoperability and data-management use cases across all formats is essential. HHS can accelerate progress by funding targeted pilot projects (for instance, through the LEAP grant program) to validate scalable, AI-powered approaches that span both structured and unstructured records.

a. Current challenges in accessing different data formats.

Many EHRs struggle to integrate unstructured content—such as scanned documents, faxed records, and free-text notes—because inconsistent standards, siloed systems, and legacy infrastructure prevent those records from appearing in the clinician's unified view.

b. Impact on patient care quality.

When critical unstructured data remains hidden, clinical decisions rely on incomplete information, driving redundant testing, delaying treatment, and increasing the risk of adverse outcomes, particularly in under-resourced rural settings.

c. Technical barriers to full data accessibility.

Transforming data into interoperable formats requires advanced solutions—like document indexing engines and standardized metadata schemas—that many systems have not implemented, and vendor platform fragmentation further impedes seamless exchange.

d. Cost or privacy implications of making all data formats interoperable.

Expanding interoperability demands substantial upfront investment in infrastructure upgrades, AI-powered tools, and staff training, and broadening access heightens privacy and cybersecurity responsibilities; however, the long-term return includes better care coordination, fewer duplicative services, and improved outcomes.

e. Priority level compared to other interoperability needs.

While exchanging structured clinical data remains the foundational priority, enabling full access to unstructured and legacy formats represents the essential next step to achieve comprehensive, patient-centric interoperability across both urban and rural healthcare environments.

PR–5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a. Patient Access API.

b. Standardized API for Patient and Population Services.

c. Provider Directory API.

d. Provider Access API.

e. Payer-to-Payer API.

f. Prior Authorization API.

g. Bulk FHIR—Do you support Group ID-based access filtering for population specific queries?

h. SMART on FHIR—Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?

i. CDS Hooks (for clinical decision support integrations).

We currently support three FHIR interfaces—all via individual queries: the Patient Access API, the Prior Authorization API, and SMART on FHIR in EHR-launched mode, which lets apps inherit clinician context. Although most integrations today use individual queries, we're beginning to pilot Bulk FHIR: we already employ FHIR Group IDs to identify cohorts for subscription-style services, even though few customers have requested bulk transactions so far.

PR–6. Is TEFCA currently helping to advance provider access to health information?

a. Please provide specific examples.

TEFCA's emphasis on a unified trust framework has encouraged broader participation by both data holders and requestors in nationwide exchange. To date, most exchanges have supported HIPAA-defined treatment workflows, demonstrating reliable, policy-backed connectivity across leading networks. Individual Access Services (IAS) exchanges have begun but remain constrained by strict IAL2/AAL2 identity-proofing requirements, and delays in finalizing SOPs for other exchange purposes have dampened interest from new entrants beyond existing national networks and Carequality participants. Providers stand to gain significantly by extending TEFCA-backed exchange to labs, pharmacies, long-term and post-acute care,

allied health, public health, and payers—especially when paired with AI tools that can analyze, summarize, and surface actionable insights from the aggregated data.

b. What changes would you suggest?

Providers will benefit tremendously from the support of TEFCA for the additional exchange purposes that have been defined and planned.

Individual Access Services (IAS): As the TEFCA community inevitably overcomes the existing barriers to IAS exchange related to some data holders requiring patient portal logon credentials via OAUTH2, IAS will dramatically improve the efficiency and lower the cost of various patient authorized data exchange workflows.

Health Care Operations (HCO): Providers are burdened by the cost and effort necessary to exchange data with various stakeholders for healthcare operations. While these exchanges are necessary and allowed under HIPAA, they are not standardized in content, format, or methodology, leading to a huge variety of workflows that providers must develop and maintain to accomplish these required exchanges. Allowing providers and their business associates to utilize standardized TEFCA exchange methods for HCO data exchanges will remove tremendous waste from our healthcare system and allow data to flow more quickly and efficiently.

Payment: The same types of benefits that will be seen in data exchanges for HCO will also improve the efficiency and security of exchanges for Payment. In this case the speed of adoption should be faster, though the financial savings may be less given the existing standardization of healthcare payment processing workflows.

PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

CMS can begin by rigorously enforcing existing Information Blocking prohibitions, clarifying applicability to all HIPAA-covered actors (including clinical labs), and expanding penalty structures to drive compliance. Beyond enforcement, CMS should sponsor a coordinated data-quality initiative—building on the VA's monitoring programs and the Sequoia Project's Data Usability Implementation Guides—to provide tools, benchmarks, and remediation workflows that dovetail with providers' existing reporting obligations. By embedding interoperability performance metrics into established clinical quality reporting and value-based payment programs, CMS can leverage familiar workflows to incentivize improvements in data completeness, consistency, and timeliness without imposing standalone burdens.

PR–8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

Current clinical quality reporting remains burdensome, slow, and costly, in part because it fails to leverage modern health IT capabilities. Adopting Bulk FHIR exports directly from certified EHR systems would streamline data collection at its source and allow CMS to calculate quality metrics on providers' behalf, reducing redundant submissions and accelerating feedback cycles.

b. In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually-purposed?

Brief incentives to shift from legacy reporting methods to automated, FHIR-based workflows would encourage rapid adoption. Embedding continuous, real-time quality reporting within certified health IT systems can align interoperability investments with existing provider performance metrics, eliminating parallel processes and driving efficiency.

c. Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

Data registries should accept Bulk FHIR feeds and return real-time quality insights directly into clinical workflows. Requiring registries to support automated data ingestion, validation, and reporting—backed by certification standards—will ensure providers receive timely, actionable feedback while simplifying ongoing compliance.

PR–9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

We recommend that CMS pursue a two-pronged approach combining targeted policy relief with programmatic incentives. First, we encourage CMS to repeal Section 510 of the Labor–HHS appropriations language, which for over 25 years has barred HHS from using federal funds to implement or adopt a unique patient identifier (UPI). Removing that prohibition would clear the path for a consistent, nationwide patient identification strategy—an essential foundation for any digital-credential framework. Second, we recommend CMS to integrate IAL2 identity proofing and multi-factor authentication requirements into the Health IT Certification Program and Promoting Interoperability rules, and offer bonus points under quality-reporting measures and Conditions of Participation for providers who adopt a single, verified credential model. To facilitate adoption, CMS can partner with ONC, ASTP, and OCR to develop practical implementation guidance—such as webinars, technical toolkits, and case

studies—that demonstrate how to add IAL2/MFA flows to existing patient portals without disrupting workflows or diminishing security.

a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

Providers will require practical implementation support, including technical integration toolkits and vendor-neutral guidance that demonstrate how to incorporate IAL2 verification and multi-factor authentication into existing portal workflows. Simplified onboarding processes, sample policy templates, and clear mapping to certification and incentive programs will reduce uncertainty and resource overhead.

b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

Requiring IAL2 identity verification and multi-factor authentication strengthens privacy by minimizing unauthorized access while offering patients a single, persistent credential across care settings. Aligning these standards with Promoting Interoperability and quality-reporting frameworks ensures that enhanced security coexists with streamlined user experiences and uninterrupted access to digital health tools.

PR–10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs):

a. What are the challenges and benefits for providers?

Providers currently face no obligation to accept or retain verified identity tokens, which means investing in token-based workflows yields little operational return. Mandating recipient systems to honor and store IAL2/AAL2 tokens would prompt modernization of authentication infrastructure, delivering stronger privacy safeguards, streamlined user management, and greater efficiency compared with legacy OAuth2-portal models.

b. How would requiring their use improve access to health information?

Enforcing digital identity verification for patients, providers, and other data requestors would bolster trust in exchange processes by ensuring that only authenticated actors access sensitive records. This increased confidence in privacy and security mechanisms would, in turn, reduce hesitation around data sharing and expand appropriate access across the ecosystem. Additionally, by anchoring every transaction to a verified identity, such a requirement would prevent—or significantly limit—damaging mismatches between individuals' records, further safeguarding patient safety and data integrity.

c. What are the potential downsides?

Completing IAL2 identity proofing can pose usability challenges for some patients and providers, necessitating dedicated support resources and fallback workflows. Organizations may need to maintain

parallel authentication paths during the transition, which could temporarily increase operational complexity and support costs.

d. What impact would mandatory credentials have on a nationwide provider directory?

Requiring all directory participants to hold verified digital credentials would raise the accuracy and reliability of endpoint listings. A directory populated with authenticated identities fosters greater confidence among consumers and systems when discovering provider services or initiating FHIR-based exchanges.

e. How could digital identity implementation improve provider data flow?

When providers prove and preserve their own IAL2 credentials, cross-jurisdictional credentialing becomes far more efficient, reducing duplication of onboarding steps for practitioners practicing in multiple states. This persistent identity model also alleviates reliance on disparate, entity-specific logins, accelerating secure data exchange.

f. Would combining FHIR addresses and identity improve data flow?

Linking verified digital identities with registered FHIR endpoint metadata in a central directory would transform service discovery. This dual-certified registry enables automated, authenticated FHIR-based queries at scale—an outcome that has proven difficult without integrated identity and endpoint management.

PR–11. How could members of trust communities 3 (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

Mandating IAL2 verification lowers unauthorized-access risk. QHINs and participants can streamline adoption by providing concise user guides and training, sharing proven onboarding workflows, and maintaining a central best-practices repository. Collaborating with CSPs to refine identity-proofing flows and support models will tailor solutions to healthcare needs, strengthening security without adding undue provider or patient effort.

PR–12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

The Information Blocking exceptions underwent lengthy public review and remain well-tailored to balance data access with legitimate privacy, security, and safety concerns. Given the minimal enforcement to date, it would be premature to remove or substantially revise these provisions. Instead, we encourage ASTP/ONC to pursue a period of robust enforcement and systematic collection of stakeholder feedback on real-world exception usage. Any future

modifications should draw on empirical insights from that enforcement phase to ensure exceptions evolve in service of both patient access and healthy market competition.

PR–13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

Linking CMS reimbursement to information-blocking compliance offers the most direct financial deterrent. Reducing payment rates for covered services—and simultaneously prohibiting balance billing for those services—would impose immediate, tangible consequences on any provider category, including labs, pharmacies, and smaller practices currently outside existing penalty frameworks. This uniform approach aligns payment incentives with timely data exchange and makes clear that withholding electronic health information carries real cost.

E. Technology Vendors, Data Providers, and Networks

TD–4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

To drive adoption of open, standards-based APIs, CMS should go beyond simply mandating FHIR endpoints and incorporate clear non-functional requirements—such as minimum performance and uptime standards—and require a designated point of contact for API registration and support. Crucially, all specifications must be precisely defined and streamlined to avoid creating a heavy compliance burden akin to Meaningful Use, ensuring that implementers can focus on innovation rather than paperwork.

TD–5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

Creating a nationwide directory of FHIR endpoints—enriched with metadata that identifies each practice, health plan, or facility—simplifies discovery for patients, providers, and payers by eliminating manual endpoint searches and enabling automated integrations. CMS (or HHS), as the agency that mandates these APIs, should build and maintain the directory. We recommend offering patient-facing API listings at no charge, while charging networks, providers, and payers a nominal fee and requiring them to register, ensuring responsible use and visibility into who accesses the directory.

TD–6. What unique interoperability functions does TEFCA perform?

a. What existing alternatives should be considered?

TEFCA establishes an official, sanctioned exchange network complete with a unified governance structure—and there are no existing alternatives that address this scope. Previous approaches succeeded only for treatment-related data sharing; lacking incentives for payment and operations workflows, they've largely failed to expand beyond clinical use, forcing organizations to contort non-treatment use cases into treatment scenarios.

b. Are there redundant standards, protocols or channels or both that should be consolidated?

Redundancies do exist, but it isn't practical to dismantle proven workflows simply for uniformity. Each protocol fulfills a distinct role and should coexist where it works best.

Example: TEFCA's push-service model excels at broad, organization-level data deliveries, whereas Direct Secure Mail remains the optimal channel for one-to-one provider-to-provider communications.

TD–7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

USCDI has standardized core clinical data elements—raising the floor for exchange across certified health IT products—but still allows variability in how fields like name and date of birth are entered, and it doesn't include operational data (e.g., appointment schedules, imaging) or guarantee unique patient identification. To bridge them, we support the MATCH IT Act of 2025, which would define a uniform patient match rate, standardize key demographic elements (to be folded into future USCDI releases), update certification requirements, and introduce voluntary performance incentives.

a. Does it contain the full extent of data elements you need?

USCDI establishes a solid baseline of core clinical elements, but it does not encompass operational data—such as real-time appointment schedules and radiological images—essential for truly patient-centric workflows and seamless care coordination.

b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

We respectfully defer from answering this question.

c. If so, would adding more data elements to USCDI add value or create scoping challenges?

Expanding USCDI to include prioritized operational datasets—particularly patient appointment and provider availability information—would deliver significant value by enabling scheduling transparency and reducing administrative overhead. Careful governance will be necessary to prevent scope creep and maintain overall usability.

TD–12. Should CMS endorse non CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

CMS should formally recognize the federally approved TEFCA network as its primary interoperability backbone—providing a unified trust framework for treatment-related data exchanges and quality reporting. Any

additional data sources or networks should meet the Common Agreement's security, privacy, and nondiscrimination requirements; demonstrate strong performance on metrics like query response times, data completeness, and latency; offer broad coverage across care settings; and operate under transparent governance that enables rapid onboarding and consistent policy enforcement.

TD–14. Regarding networks' use of FHIR APIs:

a. How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?

Health Gorilla is connected to national networks for patient data sharing, with an emphasis on clinical data. These connections support broad geographic coverage and span multiple categories of healthcare organizations.

b. How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?

Health Gorilla follows a network-of-networks model and supports connections through a variety of mechanisms. These include TEFCA-based connections using both FHIR and IHE/XCA endpoints, as well as private XCA connections to supplement overall coverage.

c. Do you interconnect with other networks? Under what frameworks (for example, TEFCA, private agreements)?

Yes, we connect under TEFCA, Carequality, CommonWell, eHealth Exchange, QHIO and other private arrangements.

TD–16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

a. Do current rules encourage scalable network participation?

The current rules do not sufficiently encourage scalable network participation. While APIs are necessary, they are not enough on their own to enable interoperability at scale. Variability in how standards are adopted and implemented makes point-to-point integration costly. A shared network model allows a participant to connect once and exchange with their preferred clinical trading partners. It also enables the network to provide common infrastructure services—such as directories, query orchestration, and translation layers—which help accelerate onboarding and reduce time to value.

b. What changes would improve alignment (for example, API unification, reciprocal access)?

A recommended change would be to leverage TEFCA and accelerate the adoption of networked, or brokered, FHIR.

TD–17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

CMS and ASTP/ONC should support the sustainability of healthcare data-sharing networks by driving demand for TEFCA-based exchange, particularly for payment and operations use cases. CMS can further strengthen network viability by requiring the use of TEFCA to fulfill data quality measures and prior authorization workflows.

TD–18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non responsiveness as well as situations that may cause a failure or unusable response?

Incomplete or inaccurate patient-matching algorithms can return false negatives or positives, preventing access to the correct record and undermining both usability and privacy. Variable configuration of C-CDA documents—where some systems only expose a basic CCD and omit other document types—limits data scope or floods recipients with irrelevant content, violating minimum-necessary principles. Finally, inconsistent data formatting and mapping can render exchanged data unusable, delaying clinical decision-making. ASTP/ONC and CMS should fund standardization of matching algorithms, tighten C-CDA content requirements, and promote real-time data-quality tools to measure and improve exchange fidelity.

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

Linking reimbursement to the adoption of modern exchange frameworks would create tangible incentives. For example, modest bonus payments for providers who implement standardized FHIR-based workflows—balanced by rate adjustments for non-adopters—would drive rapid uptake. This "carrot and stick" approach, calibrated to remain budget-neutral, aligns financial rewards with interoperable behavior.

c. Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

Payers currently escape information-blocking scrutiny despite controlling vast data flows; extending disincentives to payers would close this gap. Similarly, health-IT developers serving non-certified settings—such as EMS, dental, long-term/post-acute care, behavioral health, and niche specialty systems—operate outside ONC certification and avoid accountability. Mandating baseline interoperability standards or certification requirements for these actors would ensure comprehensive coverage under information-blocking rules.

TD–19. Regarding price transparency implementation:

a. What are current shortcomings in content, format, delivery, and timeliness?

The primary gap lies in the absence of real-time price transparency tools embedded in ordering workflows. Existing systems surface formulary alerts and basic imaging guidance but rarely deliver patient-specific cost estimates—such as copays, deductibles, or share-of-cost—when clinicians place orders or prescriptions.

b. Which workflows would benefit most from functional price transparency?

Medication ordering and prescribing workflows would see immediate gains, as would referrals and lab or imaging orders. On the patient side, price visibility during provider, pharmacy, and service shopping would enable informed decision-making and reduce sticker-shock.

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

A tool that calculates each patient's real-time out-of-pocket liability—factoring in plan benefits, deductible status, and claims history—for any proposed intervention would be most transformative, supporting shared decision-making and financial planning.

d. What would further motivate solution development?

Tying reimbursement incentives or certification requirements to the deployment of accurate, user-friendly price-transparency tools would spur vendors and providers to invest in and adopt these capabilities, ensuring solutions align with clinical workflows and patient needs.

F. Value-Based Care Organizations

VB–1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

One of our value-based care partners emphasized that stronger incentives are needed to accelerate digital health adoption in APMs like ACOs and MSSP participants. They recommend outcome-based subsidies—such as reimbursing 15–20% of SaaS costs for platforms that demonstrate a minimum 10% improvement in quality metrics or patient outcomes. With the expiration of the APM bonus and the wind-down of several VBC models due to cost overruns, future incentives should be tightly linked to independently validated improvements. They also suggest offering temporary regulatory relief, such as reduced MIPS reporting requirements, for organizations piloting certified digital solutions.

Key obstacles cited include persistent fragmentation of patient data and interoperability gaps that result in mismatches and delays in data exchange. Ongoing uncertainty around the direction and stability of VBC programs has led many providers to hesitate on new technology investments. Adoption of advanced analytics and AI remains limited—only 18% of medical groups report leveraging these tools for VBC—while the complexity of moving into risk-based contracts and the absence of clearly defined APMs for many specialties further compounds the challenge.

VB–2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

A value-based care organization we partner with recommends integrating advanced digital capabilities directly into APM requirements to drive meaningful adoption. One approach is to assign scoring weight—such as 5–10%—to the use of AI-driven tools for predictive staffing, risk stratification, care gap identification, and workflow automation. They also suggest requiring annual reporting on AI-enabled performance gains, such as reductions in documentation time or faster anomaly detection, to demonstrate value.

To encourage innovation, they propose establishing structured pilot programs within APMs for emerging technologies like real-time coding suggestion engines, contract modeling interfaces for risk negotiation, and agentic AI systems that automate care coordination and patient engagement.

Finally, they stress the need for unified, interoperable platforms. CMS should require all risk-bearing entities to meet or exceed ONC Certified Health IT (CEHRT) standards to ensure seamless integration of digital tools across care teams.

VB–3. What are essential health IT capabilities for value-based care arrangements?

a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/ normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.

Our value based partners identified several essential health IT capabilities that are critical for success under alternative payment models. Core capabilities include:

- *Care Plan Synchronization: The ability to dynamically share and update care plans across providers, payers, and patients using standards-based formats aligned with HL7 and USCDI. This ensures all stakeholders are working from a unified and current view of patient goals and care activities.*
- *Automated QPP Measure Calculation: Streamlines the process of tracking quality metrics and reporting for CMS programs, helping reduce administrative workload while ensuring compliance.*

Advanced, market-aligned capabilities include:

- *Network Performance Dashboards: Tools that provide real-time visibility into performance at the provider, practice, and network level. These dashboards support data-driven decision-making and can help optimize contract performance.*
- *AI-Assisted Documentation Workflows: Technologies such as real-time coding suggestion engines and automated care gap detection, which can reduce provider burden and improve documentation accuracy—approaches already in use by leading digital health platforms.*

b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

Additional health IT capabilities identified by our value-based care partners as critical to VBC success include:

- ***Seamless Interoperability Platforms:*** *Risk-bearing entities benefit from unified platforms that support real-time, bidirectional data flow across systems and participants. This integration reduces fragmentation, supports coordinated care, and ensures that all stakeholders operate from a consistent data foundation.*
- ***Patient Event Notification and Remote Device Integration:*** *Real-time alerts for admissions, discharges, and transfers (ADT), along with connectivity to remote monitoring devices, enable proactive outreach and timely interventions—key to managing high-risk populations effectively.*
- ***Attribution and Patient ID Matching:*** *Accurate, automated solutions for attribution and identity resolution are essential to ensure correct performance measurement, enable risk adjustment, and reduce administrative reconciliation burdens.*
- ***Data Extraction, Normalization, and SDOH Integration:*** *Automated tools that extract and normalize data from EHRs, claims, and patient-generated sources improve quality reporting and analytics. Incorporating social determinants of health (SDOH) data further supports risk stratification and targeted population health initiatives.*
- ***Patient Empowerment Tools:*** *Secure portals, mobile apps, and digital engagement tools that let patients view care plans, contribute data, and stay connected with care teams play a central role in driving adherence and outcomes.*
- ***Full-Stack Observability and Automation:*** *Continuous monitoring across workflows, coupled with intelligent automation, helps organizations identify at-risk patients earlier and improve care delivery processes over time.*

VB–4. What are the essential data types needed for successful participation in value-based care arrangements?

Key data types identified as essential for value-based care include claims history, clinical data, medication adherence, historic lab results, and social determinants of health (SDOH). These data elements collectively support accurate risk assessment, care coordination, and performance measurement.

VB–11. What specific interoperability challenges have you encountered in implementing value-based care programs?

Interoperability challenges cited include fragmented data across EHRs, payers, and care management platforms, which hinders the creation of a longitudinal patient record. Inconsistent data quality and normalization complicate attribution and risk adjustment. Smaller or rural providers often face resource constraints that limit their ability to connect to HIEs. Additionally, regulatory and contractual variations between payers and providers create barriers to seamless data exchange in multi-payer and multi-state settings.

VB–12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

A core set of standardized data elements and exchange protocols should be required to ensure consistency, while allowing APMs to extend or customize modules to meet specific clinical or operational needs. Standardized—but flexible—quality measures and reporting frameworks would support innovation while accommodating the diverse requirements of different populations and specialties.

VB–13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

To reduce provider burden while strengthening value-based care capabilities, reporting requirements across programs should be streamlined to eliminate duplication. Certification criteria should include baseline support for automated data normalization, patient matching, and real-time event notifications. Broader support for bi-directional exchange of clinical and claims data is also essential. Clear guidance and technical assistance should be made available to help smaller providers meet interoperability standards without excessive complexity or cost.

VB–14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

Digital identity credentials support value-based care by enabling accurate patient and provider identification across systems, reducing record mismatches. They simplify consent and access management, allowing care teams to securely share information in real time—critical for coordinated care. These solutions also empower patients to access and contribute to their own data, improving engagement and data quality. Greater trust and security help accelerate data sharing and the adoption of analytics and population health tools.

VB–15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

A nationwide FHIR endpoint directory would streamline real-time discovery and connectivity between providers, payers, and technology vendors, reducing onboarding friction and enabling seamless data exchange. This improved access to clinical and claims data would enhance care coordination, analytics, and timely interventions.

To be effective, the directory should include key data elements such as:

- *Organization and provider names*
- *Endpoint URLs and supported FHIR versions*
- *Available data types and services (e.g., clinical, claims, SDOH)*
- *Technical support contact information*
- *Service status and uptime indicators*
- *Certification or compliance status (e.g., TEFCA participation)*
- *Metadata on data provenance and refresh frequency*